Sunrise Diagnostics Lab

123 Wellness Avenue, Springfield | Accredited Clinical Laboratory

Patient Name:	Jane Doe	**Report Date:**	22 Aug 2026
Age / Sex:	29 / Female	**Report ID:**	SDX-88213
Referring Physician:	Dr. A. Sharma	**Sample Type:**	Venous Blood / Fasting

Panel 1 of 3: Complete Blood Count (CBC)

Parameter	Result	Normal Range	Unit	Flag
Hemoglobin	9.8	13.5 - 17.5	g/dL	LOW
White Blood Cell Count (WBC)	7.2	4.0 - 11.0	x10^3/uL	
Red Blood Cell Count (RBC)	4.1	4.2 - 5.9	x10^6/uL	LOW
Platelet Count	250	150 - 450	x10^3/uL	
Hematocrit (HCT)	34	38 - 50	%	LOW
Mean Corpuscular Volume (MCV)	88	80 - 100	fL	
Mean Corpuscular Hb (MCH)	27	27 - 33	pg	
Neutrophils	58	40 - 70	%	
Lymphocytes	32	20 - 40	%	

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Panel 2 of 3: Lipid Profile

Parameter	Result	Normal Range	Unit	Flag
Total Cholesterol	238	< 200	mg/dL	HIGH
LDL Cholesterol	162	< 100	mg/dL	HIGH
HDL Cholesterol	38	> 40	mg/dL	LOW
Triglycerides	190	< 150	mg/dL	HIGH
VLDL Cholesterol	38	5 - 40	mg/dL	
Cholesterol / HDL Ratio	6.3	< 5.0	ratio	HIGH

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Panel 3 of 3: Liver & Kidney Function

Parameter	Result	Normal Range	Unit	Flag
ALT (SGPT)	62	7 - 56	U/L	HIGH
AST (SGOT)	48	10 - 40	U/L	HIGH
Alkaline Phosphatase (ALP)	95	44 - 147	U/L	
Total Bilirubin	0.9	0.1 - 1.2	mg/dL	
Total Protein	7.1	6.0 - 8.3	g/dL	
Albumin	4.2	3.5 - 5.0	g/dL	
Blood Urea Nitrogen (BUN)	14	7 - 20	mg/dL	
Creatinine	0.9	0.6 - 1.3	mg/dL	
eGFR	98	> 90	mL/min/1.73m2	

Overall Impression

This panel shows a mild anemia pattern (low hemoglobin, RBC, and hematocrit), an elevated lipid profile (high total cholesterol, LDL, and triglycerides with low HDL), and mildly elevated liver enzymes (ALT and AST). Kidney function markers are within normal limits. These findings are consistent with a combination of nutritional and metabolic factors and warrant clinical correlation.

Category	Flagged Parameters	Direction
Blood Count	Hemoglobin, RBC, Hematocrit	Low
Lipid Profile	Total Cholesterol, LDL, Triglycerides, Chol/HDL Ratio	High
Lipid Profile	HDL Cholesterol	Low
Liver Function	ALT, AST	High

Recommendation: Correlate clinically with patient history and symptoms. Consider iron studies, dietary assessment, and a repeat lipid panel after lifestyle modification. Not a substitute for professional medical diagnosis.